

March 30, 2026

Adrabetadex Access, Reimbursement, and Provider Economics Stakeholders

Internal narrative brief

Team,

Re: Adrabetadex provider net cost calculator narrative brief

The net cost calculator has to do more than estimate an allowed amount. Across the source set, adrabetadex launches at a working WAC of \$39,000 per 900 mg vial dosed every two weeks, or \$1,014,000 in annual gross drug WAC before administration cost, wastage, denials, or financing drag. The provider concept brief already shows why a simple reimbursement estimate is not decision-useful: commercial buy-and-bill at 100% of WAC produces zero drug spread, the non-340B commercial operating case turns negative once labor, denial drag, and payment-speed cost are applied, and 340B economics are materially better because acquisition cost drops while payment benchmarks stay linked to the outpatient file. The combined implication is that the model must translate coverage and channel mechanics into provider margin and cash exposure by site and payer scenario, not just into a single net price view (Adrabetadex Provider Net Cost Model Concept Brief, pp. 1, 4-6; Adrabetadex U.S. Launch Background and Core Assumptions, pp. 2-3).

The reimbursement landscape also means the calculator has to behave like a launch-execution engine. The reimbursement addendum explains that adrabetadex will enter a medical-benefit market managed through prior authorization, specialist prescribing, combination restrictions, white-bagging, site-of-care review, and claim-level documentation. It further ties early hospital outpatient payment predictability to HCPCS and OPPS pass-through readiness, with the earliest practical pass-through effective date on January 1, 2027 if the CMS MEARIS package is complete for the September 2026 cycle. For modeling purposes, that means the calculator needs distinct branches for pre-code miscellaneous billing, product-specific HCPCS billing, and pass-through-active billing, because each state changes denial risk, claim manual work, and payment timing (Adrabetadex U.S. Reimbursement Landscape, HCPCS, and Pass-Through, pp. 1-3; Adrabetadex Provider Net Cost Model Concept Brief, pp. 3-4).

Aetna's published policy is useful because it shows what commercial friction looks like in operational terms. In Clinical Policy Bulletin 0442, Aetna states that precertification of enzyme replacement drugs is required for participating providers and members in applicable plan designs and directs providers to a Statement of Medical Necessity process. The same bulletin states that Aetna's Site of Care Utilization Management Policy applies to enzyme replacement drugs. That combination is commercially important because it signals two separate provider burdens before a clean claim exists: the center has to assemble a medical-necessity record that supports approval, and it has to do so in a site that the payer is willing to reimburse under its specialty drug infusion rules (Aetna Clinical Policy Bulletin 0442, p. 2).

Aetna also shows that analogous rare-disease therapies are managed through exact eligibility, administration, and reauthorization gates. For Brineura, Aetna excludes treatment when dosage exceeds 300 mg every other week or when the member has acute intraventricular



device-related complications or a ventriculoperitoneal shunt; initial approval requires confirmed diagnosis and administration by, or under the direction of, a physician knowledgeable in intraventricular administration; and continuation requires no loss of ambulation or a slowed loss of ambulation from baseline. The same bulletin repeatedly states that uses outside listed criteria are considered experimental, investigational, or unproven. Adrabetadex will not inherit those exact criteria verbatim, but the policy is still direct evidence that a major commercial payer operationalizes rare neurologic and lysosomal therapies through dosage controls, specialist requirements, exclusion logic, and function-based continuation standards rather than through open-ended medical-benefit coverage (Aetna Clinical Policy Bulletin 0442, p. 6).

That commercial evidence should change what the calculator is designed to capture. The tool should not stop at acquisition cost versus expected payment. It should quantify the cost of payer friction that providers experience while attempting to access reimbursement. In practical terms, the model needs to recognize that white-bagging can remove drug spread entirely, non-340B buy-and-bill can remain underwater without added support, 340B carve-in and carve-out decisions change both acquisition cost and Medicaid duplicate-discount compliance work, and Medicaid fee-for-service versus managed care changes claim mechanics and NDC readiness requirements. Those are not side assumptions; they are the main drivers of whether a center can absorb launch volume without damaging working capital (Adrabetadex Provider Net Cost Model Concept Brief, pp. 2-6; Adrabetadex U.S. Reimbursement Landscape, HCPCS, and Pass-Through, pp. 1-4).

At a minimum, the calculator should include the following modules and outputs:

- Scenario architecture for commercial buy-and-bill, commercial white-bagging, hospital outpatient versus physician office, Medicaid fee-for-service, Medicaid managed care, 340B carve-in, 340B carve-out, and the limited dual-eligible pathway.
- Acquisition logic that can flex between WAC, estimated 340B ceiling price, and manufacturer support assumptions, with explicit visibility into per-dose and annualized cash outlay.
- Payment logic that distinguishes miscellaneous-code billing, product-specific HCPCS billing, pass-through-active hospital outpatient billing, and payer-specific commercial reimbursement views below WAC.
- Coverage-friction fields for precertification status, documentation completeness, site-of-care approval, specialist-administration requirements, dose or quantity-limit edits, and reauthorization timing tied to baseline and follow-up clinical measures.
- Claim-performance inputs for denial probability, resubmission effort, NDC capture quality, days to payment, and financing cost while high-dollar inventory is outstanding.
- Provider-operating inputs for procedure revenue, labor time, pharmacy handling, and center-level assumptions that differ between hospital outpatient departments and office infusion settings.
- Outputs that show drug spread, net operating margin, annualized contribution, break-even manufacturer support, and cash exposure under each channel so the provider can see not only whether a claim may be covered, but whether the treatment pathway is economically workable.

The reason to combine the three Adrabetadex documents into one brief is that each one answers a different part of the provider question. The launch background establishes the



commercial assumptions and the managed-access environment, the reimbursement addendum explains how coding and pass-through mechanics create or relieve launch friction, and the provider concept brief shows where economics break once acquisition, denial, and payment-speed realities are applied. Aetna's policy bulletin then grounds those abstractions in the actual friction a national commercial payer imposes today. Taken together, the documents support a clear design principle: the provider net cost calculator should be built as a reimbursement-readiness and cash-risk model, not as a static net price tool.

Sources used in this brief:

- Adrabetadex: Provider Net Cost Model Concept Brief, supplied PDF dated March 30, 2026.
- Adrabetadex: U.S. Reimbursement Landscape, HCPCS, and Pass-Through, supplied PDF dated March 30, 2026.
- Adrabetadex U.S. Launch Background and Core Assumptions, supplied PDF dated March 30, 2026.
- Aetna Clinical Policy Bulletin 0442, Lysosomal Storage Disorder Treatments, extracted from the supplied Aetna PDF dated March 30, 2026, with cited passages from pages 2 and 6.

Sincerely,

Codex

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